

Postpartum - High Blood Pressure - Female (Child)

Algorithm 1428 | Child (12-17 years) | Gender at birth: Female

Definition

- Postpartum high blood pressure assessment after birth; a reported reading must be confirmed and interpreted by a clinician using an approved obstetric blood-pressure pathway.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the blood pressure reading, if measured?
2. [DURATION] How many days/weeks since delivery?
3. [YES_NO] Any headache, vision changes, or swelling?
4. [YES_NO] If the patient is under 18, can they speak privately now, and do they say they feel safe with the accompanying adult?
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Disposition level 70

Is high blood pressure the primary concern without recent childbirth?

The authored Blood Pressure - High protocol is adult-only and cannot receive an adolescent redirect. Arrange governed in-person clinical review without applying a permissive alias.

1. Disposition level 100

Along with high blood pressure, is there a severe headache, vision problems, pain below the ribs, sudden swelling of the face/hands/feet, feeling very unwell, or vomiting?

NHS.UK pre-eclampsia guidance lists these as warning signs requiring immediate evaluation, which can develop in the days or weeks after birth.

1. Disposition level 70

Has a clinician said the postpartum blood pressure is high, or is there a new home reading above the patient's clinician-agreed range, without the emergency features above?

The source confirms high blood pressure can be an early sign of postpartum pre-eclampsia but does not provide a complete telephone threshold or measurement protocol; the reading needs prompt confirmation under an approved obstetric pathway.

Advice

Emergency postpartum pre-eclampsia precautions

- Call Qatar emergency services on 999 now. Do not drive yourself. Tell the call handler that the patient gave birth recently. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening headache or vision changes.

Urgent postpartum blood pressure review

- Arrange prompt blood-pressure confirmation through the locally approved Qatar maternity pathway; the exact non-emergency destination and measurement thresholds remain GOVERNANCE_REQUIRED.
- Call Back If: headache, vision changes, or swelling develop.

First aid

- Call Qatar emergency services on 999 now. Do not drive yourself. Tell the call handler that the patient gave birth recently. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Pre-eclampsia", <https://www.nhs.uk/conditions/pre-eclampsia/> (page last reviewed 23 March 2026)
- explicitly covers postpartum onset

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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